
Neurodivergent Care App

Complete Resource Pack, Script Library & Content Work Plan

Caregiver Education · Children's Animated Series · Daily Check-In Framework
Developed for the Kenyan clinical & community context

Version 1.0 · 2025

This document is a complete working plan for app content development. It covers the full domain taxonomy, video script library for caregiver-facing content, the full children's animated series with episode-by-episode scripts, the daily check-in specification, and the clinical report framework. Each section is production-ready.

Contents

Part 1 — Overview & Domain Framework

- 1.1 App vision & purpose
- 1.2 Full content domain taxonomy
- 1.3 Age band framework
- 1.4 Content production overview

Part 2 — Daily Check-In Specification

- 2.1 Caregiver daily check-in (7 questions)
- 2.2 Child mood-motor check (ages 5+)
- 2.3 Weekly clinical summary — field definitions

Part 3 — Caregiver Video Script Library

- Domain 1: Sensory Processing (C-01 to C-04)
- Domain 2: Motor Development (C-05 to C-06)
- Domain 3: Communication & Language (C-07 to C-09)
- Domain 4: Emotional Regulation (C-10 to C-13)
- Domain 5: Sleep & Rest (C-14 to C-16)
- Domain 6: Daily Living & Routines (C-17 to C-19)
- Domain 7: Social Skills (C-20 to C-21)
- Caregiver Hub (C-22 to C-24)

Part 4 — Children's Animated Series

- Series A: My Feelings Are Big (ages 4–7) — 6 episodes
- Series B: Friends at School (ages 5–8) — 6 episodes
- Series C: My Body Knows (ages 3–6) — 5 episodes
- Series D: My Day, My Way (ages 4–8) — 5 episodes

Part 5 — Clinical Report Template

- 5.1 Weekly automated summary field definitions
- 5.2 Multidisciplinary team routing guide

Part 1 — Overview & Domain Framework

1.1 App Vision & Purpose

This app serves neurodivergent children and their caregivers operating within the Kenyan healthcare and community context. It bridges the gap between infrequent specialist appointments by equipping families with evidence-informed skills, structured daily tracking, and shareable clinical summaries that eliminate the need for caregivers to repeat their child's history at every new appointment.

Two parallel tracks run throughout the app: a caregiver-facing resource library of short filmed videos and downloadable guides, and a children's-facing animated series that teaches emotional, social, and sensory skills through age-appropriate storytelling. A daily check-in module ties both tracks together, generating a rolling clinical record that can be shared with occupational therapists, neurologists, speech and language therapists, and psychiatrists.

1.2 Full Content Domain Taxonomy

All content is organised into seven clinical domains plus a dedicated caregiver hub. Each domain has resource pack modules (short videos + downloadable guides), a section of the daily check-in, and one or more animated series episodes aimed at children.

Sensory Processing

- Sensory corner setup for home
- What is a sensory diet — and how to build one
- Proprioceptive (heavy work) activity menu
- Tactile sensitivity — management strategies
- Auditory sensitivity — noise management at home and school
- Visual sensitivity — lighting and clutter reduction
- Vestibular play and movement breaks
- Oral motor tools and oral sensory needs
- Building a sensory toolkit for school
- Interoception — helping your child read internal body signals

Motor Development

- Fine motor skill ladder — from tearing to writing
- Gross motor circuit ideas for small home spaces
- Bilateral coordination games using local materials
- Handwriting readiness — what to build before the pencil
- Oral motor exercises for feeding and speech clarity
- Body awareness games
- Embedding motor work into daily routines (not as extra therapy)

Communication & Language

- Understanding echolalia — what it is and how to use it
- Introduction to AAC for families
- Low-tech visual communication boards from local images
- Non-verbal behaviour dictionary for caregivers
- Social scripts for greetings, refusals, and requests
- Turn-taking and early conversation skills
- Conversation frameworks for siblings
- Protesting and requesting — how to expand communication functions

Emotional Regulation

- Meltdown vs tantrum — the clinical difference
- The five zones of regulation
- Co-regulation — why the caregiver's nervous system leads
- Box breathing and body-based calming tools
- Building a calming kit at home
- Emotional coaching language scripts for caregivers
- Recognising early dysregulation — what to watch for
- After the storm — repair and reconnection

Sleep & Rest

- Why neurodivergent children often have disrupted sleep
- Sleep hygiene adapted for Kenyan home contexts
- Building a bedtime routine visual board
- Managing sleep regression by age
- Night terrors vs night waking — what to do
- Safe weighted blanket use and alternatives

Daily Living & Routines

- How to build a visual schedule
- First/then boards for all ability levels
- Transition warnings — the 5-minute, 2-minute, now system
- Food selectivity — why it's sensory, not defiance
- Toileting training for children with sensory differences
- Dressing and self-care routine building
- Community preparation — church, market, hospital visits

Social Skills

- Peer play skills by age band
- Friendship scripts and greeting routines
- Understanding and teaching personal space
- Interpreting facial expressions using local norms
- Joining a group game — step-by-step scripts
- Friendship maintenance for school-age children
- Peer education pack — what to say to classmates

Caregiver Hub

- Progressive muscle relaxation — guided session
- Box breathing for caregiver regulation
- Caregiver grief and ambiguous loss — psychoeducation
- Preventing and managing caregiver burnout
- Building a multidisciplinary team in Kenya
- What to bring to an OT or neurology appointment
- How to read a therapy report
- Advocating for your child in school — scripts

1.3 Age Band Framework

All content — caregiver videos, animated episodes, and check-in tools — is tagged to one or more of the following age bands. Animated series episodes are produced per band. Caregiver videos are relevant across bands but include age-specific guidance within each script.

Band	Age Range	Developmental Focus	Key Needs
Toddler	18 months – 3 yrs	Sensory, sleep, early communication	Parent-led, very visual, minimal language
Preschool	3 – 5 yrs	Regulation, routines, early social play	Simple scripts, repetition, song-based
Early school	5 – 8 yrs	Social skills, classroom readiness, friends	Story-led, relatable characters, humour
Middle school	8 – 12 yrs	Self-advocacy, friendships, emotions	Peer-level, realistic scenarios, agency
Teen (future)	12 – 17 yrs	Identity, independence, mental health	Peer-voice, co-produced content

1.4 Content Production Overview

The following volumes are planned for the initial release:

Content Type	Volume (v1)	Format	Language
Caregiver resource videos	24 videos	Filmed, 55–75 sec each	English + Kiswahili
Caregiver downloadable guides	24 PDF guides	1 per video, printable	English + Kiswahili

Animated children's episodes	22 episodes	2D animation, 3–5 min each	English + Kiswahili
Daily check-in module	1 module	App screen, daily use	English + Kiswahili
Weekly clinical summary	Auto-generated	PDF export per child	English (clinical)

Part 2 — Daily Check-In Specification

The daily check-in is the data spine of the app. It takes under three minutes, is visual enough for users with limited literacy, and feeds an automated weekly summary that can be shared directly with any specialist. Each question maps to a clinical domain.

2.1 Caregiver Daily Check-In — 7 Questions

#	Question	Response Type	Clinical Domain
1	How did your child sleep last night?	3 icons: easy / long time / very disrupted	Sleep
2	What was your child's mood when they woke up?	5 face emoji scale (1–5)	Emotional Regulation
3	How was eating today?	3 options: ate well / selective / refused	Daily Living
4	Did any sensory difficulty happen today?	4 levels: none / mild / significant / severe (meltdown)	Sensory
5	One thing that went well today	Short open text (1–2 sentences)	All domains
6	One thing that was hard today	Short open text (1–2 sentences)	All domains
7	How are YOU coping today? (caregiver)	1–5 star scale	Caregiver wellbeing

2.2 Child Mood-Motor Check (ages 5+)

A separate three-question visual check-in designed for the child to complete with or without caregiver support. Uses large tap-on-screen picture buttons. No reading required.

#	Question	Picture Options
1	How does your body feel right now?	Calm body (blue) / Tight body (orange) / Bouncy body (yellow)
2	How big are your feelings?	Small fish / Medium fish / Big whale
3	What do you need right now?	Quiet time / Movement / A hug / A snack / More options button

2.3 Weekly Clinical Summary — Field Definitions

Every Sunday the app auto-generates a one-page PDF summary of the past seven days. The caregiver can share this directly with their OT, neurologist, SALT, or psychiatrist via WhatsApp or email. Fields are as follows:

Field	What it shows	Flag threshold
Sleep quality trend	7-day bar chart of sleep scores	3+ 'very disrupted' nights → amber flag
Sensory difficulty frequency	Count of days at each severity level	2+ severe days → red flag

Mood on waking trend	7-day line of mood scores	Consistent 1–2 for 5+ days → flag
Caregiver coping trend	7-day line of caregiver score	Score 1–2 for 4+ days → flag
Eating pattern	Summary of eating responses this week	3+ refusal days → flag
Caregiver free text (compiled)	All open text entries chronologically	Clinician reads in full
Child check-in (if completed)	Summary of body/feeling/need responses	Clinician reviews patterns
Red flag summary	Automated plain-English alert paragraph	Any flagged field included here

Part 3 — Caregiver Video Script Library

Each script below is production-ready. Videos should be filmed with a warm, direct presenter in a Kenyan home or clinical setting. All scripts run 55–75 seconds at a natural speaking pace. Each video ends with a single action prompt. Kiswahili versions should be adapted (not directly translated) to preserve natural expression.

Production notes: Use natural light. No white-coat aesthetic — presenter should be warm and approachable. Kenyan household props where relevant (chapati reference in box breathing, local food in feeding video). Closed captions on every video.

Sensory Processing

C-01 — What is a sensory diet?

Code: C-01 | **Domain:** Sensory Processing | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~60 sec

[Open on caregiver at home with child nearby — relaxed, warm]

You've probably heard the phrase 'sensory diet' — and no, it has nothing to do with food.

A sensory diet is a personalised plan of movement and touch activities that help your child's nervous system stay calm and ready to learn.

Think of it this way: some children need more sensory input than their environment gives them — they're always touching things, crashing into furniture, seeking big experiences.

Others are overwhelmed by too much input — loud sounds, scratchy clothes, or busy spaces shut them down.

A sensory diet gives them the right amount of input, at the right times, before the nervous system gets overwhelmed.

Simple activities count: jumping on a mattress before school, carrying a heavy bag, playing with playdough, or a firm bear hug. Your occupational therapist can help build one specific to your child.

Start with two or three activities woven into your morning. Track what helps in the daily check-in — that data will let your OT adjust the plan over time.

[End card: 'Try one activity before school this week. Track how it goes.']

Sensory Processing

C-02 — Setting up a sensory corner at home

Code: C-02 | **Domain:** Sensory Processing | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show a simple corner of a room being arranged — nothing expensive]

A sensory corner is a small, defined space in your home where your child can go to calm

down, regulate, and reset. You do not need a big home or expensive equipment.

Choose a corner — not the bedroom if possible, somewhere the whole family can see it's a safe space, not a punishment. Mark it gently: a small mat, a cushion, a low shelf.

Stock it with things specific to your child. For a child who seeks heavy input: firm cushions to squeeze, a weighted lap pad, a stretchy band on the chair. For a child overwhelmed by sensation: soft fabric, noise-cancelling earmuffs, dim lighting, a favourite small object.

Add something for the mouth if your child has oral sensory needs — a chew toy, a crunchy snack option, or a water bottle.

Introduce it when your child is calm, not in crisis. Practice going there together first. Over time, your child will learn to use it independently.

[End card: 'Build your corner this week. Involve your child in choosing what goes in it.']

Sensory Processing

C-03 — Proprioceptive activities — heavy work at home

Code: C-03 | **Domain:** Sensory Processing | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~60 sec

[Show child doing wall push-ups, carrying groceries]

Proprioception is your child's sense of their own body in space — the feedback from muscles and joints. Many neurodivergent children are under-responsive to this sense, which is why they seek out crashing, jumping, pushing, and squeezing.

The good news: you can provide this input deliberately, before the day gets hard.

We call these activities 'heavy work' because they give the muscles and joints a significant workout.

Ideas that cost nothing: wall push-ups, carrying a loaded bag, pushing or pulling furniture with supervision, wheelbarrow walking — holding your child's ankles while they walk on their hands, animal walks like bear crawls, and rough-and-tumble play.

Heavy work is most effective when done before a demand — before school, before a meal, before a transition. It fills the proprioceptive tank and buys your child a window of calm.

[End card: 'Try 5 minutes of heavy work before your child's hardest time of day.']

Sensory Processing

C-04 — Auditory sensitivity — practical strategies

Code: C-04 | **Domain:** Sensory Processing | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~60 sec

[Show a child covering their ears in a busy market — then calm at home]

If your child covers their ears, becomes distressed in crowds, or refuses to enter loud spaces, they may have auditory sensitivity — a nervous system that processes sound more intensely than average.

This is not willfulness. Their ears genuinely receive sound differently. Certain pitches, volumes, or unexpected sounds can cause real pain or panic.

Practical strategies: noise-cancelling headphones or ear defenders are a genuine tool, not a crutch. Use them in noisy environments and remove them gradually as your child adjusts. Warn before loud events — a two-minute alert makes a difference.

At home: reduce unnecessary background noise during meals and homework. Choose quieter environments for outings where possible. Give your child a signal — like a hand squeeze — to tell you it's getting too loud without having to shout in a crowd.

Respect the ears. Your child's response is neurological, not dramatic.

[End card: 'Identify your child's top two loud triggers this week.']

Motor Development

C-05 — Fine motor skills — building the foundation

Code: C-05 | **Domain:** Motor Development | **Audience:** Caregivers | **Age:** 2–8 yrs | **Duration:** ~60 sec

[Show child tearing paper, threading beads, then eventually writing]

Fine motor skills are the small, precise hand movements your child needs for writing, buttoning, eating, and dozens of daily tasks. Many neurodivergent children have delays in this area, and that is something we can work on step by step.

The progression matters: you cannot start with a pencil. Start with the hands.

For ages two to four: tearing paper, squeezing playdough, picking up small objects.

For ages four to six: threading large beads, using child scissors, manipulating pegs and buttons. For ages six to eight: smaller beads, pencil grip activities, lacing boards, and controlled cutting.

Embed this into play, not homework. Playdough at the table, tearing old newspapers for an art project, sorting beans into cups — these build the same muscle memory.

If writing is a significant struggle, tell your OT. Adapted pencil grips and alternative writing tools can remove a daily battle.

[End card: 'Start one fine motor activity today — playdough counts.']

Motor Development

C-06 — Handwriting readiness — what to build first

Code: C-06 | **Domain:** Motor Development | **Audience:** Caregivers | **Age:** 4–8 yrs | **Duration:** ~65 sec

[Show child doing large arm movements on chalkboard, then fingers]

Before a child is ready to write with a pencil on paper, they need three things: core strength to sit upright, shoulder stability to control the arm, and fine motor control in the hand. If any of these is missing, handwriting will always be a battle.

Core strength: does your child slump at the table after a few minutes? Build this with seated balance activities — sitting on a cushion, reaching across the midline, or animal walks that require holding a position.

Shoulder stability: large arm movements come before small ones. Let your child draw on a vertical surface — a wall, a chalkboard, a window with washable markers.

Moving the whole arm before the wrist and fingers is developmentally correct.

Pre-writing shapes: before letters, practice lines, circles, crosses, and waves.

A child who cannot copy a plus sign reliably is not ready for letter formation.

Talk to your OT before drilling handwriting. Drill without readiness builds bad patterns that are harder to undo later.

[End card: 'Let your child draw on a wall surface this week. Big movements first.']

Communication & Language

C-07 — Understanding echolalia

Code: C-07 | **Domain:** Communication & Language | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show child repeating a phrase from a cartoon, then using a similar phrase to communicate]

Echolalia is when a child repeats words, phrases, or entire sentences they have heard before — from you, from cartoons, from songs. Many families are worried by it.

In most cases, echolalia is not a problem. It is communication.

Immediate echolalia is when your child repeats what you just said. This is often processing — the brain is using the repetition to make sense of the input.

Delayed echolalia is when they use a phrase from something they heard earlier.

'This is a disaster' said when they drop their cup may be from a cartoon — but it means 'I am upset.' The language is borrowed, but the communication is real.

Do not try to stop echolalia. Instead, respond to the communicative intent behind it. If they say 'time to go' when they want to leave — say 'yes, time to go'

and help them leave. You are confirming the function and building the connection between the phrase and the situation.

Over time, with support, many children develop more flexible language alongside their echolalic phrases.

[End card: 'Listen for the intent behind the phrase, not just the words.']

Communication & Language

C-08 — Non-verbal behaviour — a parent's dictionary

Code: C-08 | **Domain:** Communication & Language | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show different child behaviours with caregiver interpreting warmly]

Not all communication uses words. Neurodivergent children often communicate through behaviour, and learning to read that behaviour is one of the most powerful skills a caregiver can build.

Turning away or covering eyes: often means 'too much — I need a break from this. interaction or this environment.'

Approaching and touching something repeatedly: often curiosity, interest, or a request. 'I want this' or 'I want to do this.'

Self-stimulatory behaviours — rocking, hand-flapping, humming: usually regulation. The body is managing arousal levels. These are not behaviours to stop unless they cause harm.

Pushing food away, gagging, or refusing a chair: often sensory communication. Something about this experience is overwhelming.

Running to the door: could be a request to go somewhere, or a way to escape something overwhelming. Context tells you which.

The question to ask with any behaviour: what is my child trying to tell me?

[End card: 'Pick one behaviour this week and ask — what could this be communicating?']

Communication & Language

C-09 — Introduction to AAC for families

Code: C-09 | **Domain:** Communication & Language | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~70 sec

[Show caregiver using a picture board with child naturally]

Augmentative and alternative communication — AAC — is any tool that helps a child communicate beyond or instead of speech. Picture boards, symbol-based apps, choice cards, and gesture systems are all forms of AAC.

The most common question families ask is: 'Will AAC stop my child from talking?'
The research says clearly — no. AAC supports speech development. It does not replace it. Giving your child more ways to communicate reduces frustration and actually creates more language opportunity.

You do not need an expensive device to start. A homemade picture board with laminated photos of your child's world — their favourite foods, places, people, and activities — is an effective AAC tool.

Model the system yourself first. Point to the picture of water when you offer water. Touch the picture of 'finish' when an activity ends. You are showing your child how the system works before asking them to use it.

Talk to your speech and language therapist about the right system for your child. If you do not have one, use the app's specialist directory to find support in Kenya.

[End card: 'Make three picture cards this week — your child's favourite things.']

Emotional Regulation

C-10 — Meltdown or tantrum — what's the difference?

Code: C-10 | **Domain:** Emotional Regulation | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Open calm — no dramatisation of behaviours in background]

When your child has a big reaction — screaming, hitting, falling to the floor — it can be hard to know what is happening. Is this a tantrum? A meltdown?

The difference matters because the response that helps is completely different.

A tantrum is goal-directed. The child is communicating: I want something, or I want to avoid something. They are still aware of you. They may look at your reaction. If the goal goes away, the tantrum often stops.

A meltdown is neurological. The child's nervous system has been overwhelmed — by too much sensory input, too many demands, or too much change — and they have lost the ability to regulate. They are not performing. They cannot stop.

During a meltdown: reduce stimulation, stay nearby and calm, speak very little.

Use a low, slow voice. Do not try to reason, teach, or apply consequences.

Just be a calm presence.

After it passes — and only then — connection and gentle conversation can happen.

[End card: 'Notice which one it is next time. It changes what you do first.']

Emotional Regulation

C-11 — Co-regulation — why your calm is the tool

Code: C-11 | **Domain:** Emotional Regulation | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show caregiver deliberately slowing breath before approaching child]

There is a concept in neuroscience called co-regulation. It means that the nervous system of a child — especially a dysregulated child — responds directly to the nervous system of the adult near them.

When you are calm, your child's brain has something to sync with. When you are flooded with stress, your child's dysregulation escalates to match yours.

This is not your fault. It is biology. But it does mean that the most powerful regulation tool you have is your own body.

Before you approach a meltdown: slow your own breath first. Two or three deep exhales. Relax your shoulders. Soften your face and voice. You are not performing calm — you are trying to genuinely access it, because your child can feel the difference.

This is hard when you are exhausted, overwhelmed, or also dysregulated. The caregiver check-in in this app tracks your own coping because it matters clinically.

You cannot pour from an empty cup, and you are allowed to put yours down for a moment.

[End card: 'Practice one slow exhale before your next hard moment.']

Emotional Regulation

C-12 — Box breathing — a tool for all ages

Code: C-12 | **Domain:** Emotional Regulation | **Audience:** Caregivers + children | **Age:** 4+ yrs | **Duration:** ~70 sec

[Show caregiver teaching a child — both doing it together]

Box breathing is a simple, evidence-based technique that activates the parasympathetic nervous system — the system that tells the body it is safe to calm down.

Here is how it works. We breathe in four counts, hold four counts, breathe out four counts, and pause four counts. Like drawing the four sides of a box.

Let's try together. Breathe in — two, three, four. Hold — two, three, four.

Out — two, three, four. Pause — two, three, four.

For younger children, use a picture. Breathe in like you are smelling chapati.

Breathe out like you are blowing it cool. Make it concrete.

Practice this when everyone is calm — not first in a crisis. Practised breathing

is available in a crisis. Unlearned breathing is not.

Do it together at bedtime, in the car, or after school. Your child learns it by watching you use it, not just by being told to.

[End card: 'Practice one box breath together tonight at bedtime.']

Emotional Regulation

C-13 — After the storm — repair and reconnection

Code: C-13 | **Domain:** Emotional Regulation | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~60 sec

[Show quiet, gentle scene — caregiver sitting near child after a hard moment]

When a meltdown ends, both your child and you are likely to be depleted. The nervous system has gone through something significant. What happens next matters.

Repair is the process of reconnecting after a rupture — and it is something you can initiate even if you handled the meltdown imperfectly.

Give your child time to come back to themselves first. Don't rush into hugs or conversation. Watch for the moment they seem ready — they may make eye contact, move towards you, or relax their body.

When that moment comes: low voice, simple words. 'That was hard. I am here.'

Physical closeness if your child accepts it. No discussion of behaviour, no consequences, no 'next time' conversation — not yet.

Later — hours later, or the next day — is when calm, brief teaching can happen.

But repair first. Connection is the foundation that makes everything else possible.

[End card: 'After the hard moment — reconnect first. Teach later.']

Sleep & Rest

C-14 — Building a consistent bedtime routine

Code: C-14 | **Domain:** Sleep & Rest | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~60 sec

[Show gentle evening scene — dim light, calm activity]

The neurodivergent brain often struggles with transitions — and the biggest transition of the day is going from wakefulness to sleep. A consistent bedtime routine is one of the highest-impact tools you have.

The goal is a predictable sequence of events that signals to the brain: sleep is coming. Do the same steps, in the same order, every night.

A simple structure: dim the lights about thirty minutes before bed — this supports melatonin production. Then one physical activity like a warm bath or gentle stretching.

Then something calm — a story, quiet music, or sensory brushing. Then into bed.

Keep screens off at least forty-five minutes before sleep. Blue light significantly delays melatonin in neurodivergent children.

If your child shares a bed with you, the same cues apply — it is the sequence that teaches the brain, not the location.

[End card: 'Write down your three-step bedtime sequence tonight and stick to it.']

Sleep & Rest

C-15 — Understanding sleep regression

Code: C-15 | **Domain:** Sleep & Rest | **Audience:** Caregivers | **Age:** 0–8 yrs | **Duration:** ~60 sec

[Show tired caregiver — acknowledge it directly]

Sleep regression is when a child who was sleeping reasonably well suddenly stops sleeping. It is exhausting, and it is very common in neurodivergent children.

Regressions often happen at developmental transition points: around 18 months, at age two to three, at school entry, and again around ages six to seven. They can also be triggered by change — a house move, a new sibling, a school transition, illness, or a change in routine.

What to do: maintain the routine even when it is not working immediately. Consistency is the active ingredient — the brain is recalibrating, and it needs the signal to remain steady. Expect two to four weeks before improvement.

What not to do: don't start new sleep habits during a regression that you are not prepared to maintain. If you bring your child into your bed as a temporary fix, expect that to become permanent until you actively change it.

[End card: 'Log sleep quality in the daily check-in — it helps track the pattern.']

Sleep & Rest

C-16 — Weighted blankets — how and when to use them

Code: C-16 | **Domain:** Sleep & Rest | **Audience:** Caregivers | **Age:** 2+ yrs | **Duration:** ~55 sec

[Show a weighted blanket being used correctly — calm child]

Weighted blankets use deep pressure — firm, even weight across the body — to activate the parasympathetic nervous system and promote a sense of calm. They are a helpful sensory tool for many neurodivergent children, particularly at bedtime.

The standard weight guide is ten percent of the child's body weight, plus one kilogram. So for a twenty-kilogram child, a three-kilogram blanket. Never go heavier than this.

Safety rules: the child must be able to remove the blanket themselves. Never use on children under two, or on any child who cannot push it off independently. Do not cover the head.

If you cannot access a commercial weighted blanket in Kenya, a heavy cotton blanket or a kanga folded several times can provide similar pressure. The key is even, full-body weight — not heavy on one spot.

[End card: 'Try ten minutes with a heavy blanket at bedtime this week.']

Daily Living & Routines

C-17 — First/then — the simplest tool in your toolkit

Code: C-17 | **Domain:** Daily Living & Routines | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~55 sec

[Show caregiver making a quick two-column card at the table]

If there is one tool that works across almost every age, every ability level, and every situation — it is the first/then board.

First we do this. Then we get that. That is the whole system.

'First shoes on. Then we go to the park.' 'First bath. Then story.'

'First eat two bites. Then the tablet.'

Why does it work? Because it makes the near future visible and predictable.

Neurodivergent children often resist transitions because they cannot see what comes next. The first/then board shows them there is something good after the hard part.

Keep it to two steps only. Use it before the demand, not during resistance.

Make the 'then' something your child actually wants.

It is not a bribe. It is a map.

[End card: 'Use first/then for your child's hardest transition today.']

Daily Living & Routines

C-18 — Visual schedules — how to build and use one

Code: C-18 | **Domain:** Daily Living & Routines | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show a simple handmade schedule on a wall — photos and drawings]

A visual schedule is a sequence of pictures, symbols, or words that shows your child what their day — or part of their day — looks like in order.

Why visuals? Language is processed in real time and then disappears. A picture stays there. Your child can look at it again and again. They can see what is done,

what is next, and what is coming. That predictability reduces anxiety significantly.

You do not need to buy anything. Photograph your child's actual routine — their actual breakfast bowl, their actual school bag, their actual bedroom — and arrange the photos in order on a strip on the wall. Laminate if you can. Use velcro so you can move items.

For non-readers: photos work. For early readers: one word under the photo.

For older children: a written list works too.

Teach your child to check the schedule, not to check you. That is the goal: the schedule answers the question 'what's next' so you don't have to every time.

[End card: 'Build a three-step morning schedule with photos this weekend.']

Daily Living & Routines

C-19 — Food selectivity — it's sensory, not naughty

Code: C-19 | **Domain:** Daily Living & Routines | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show caregiver serving meal calmly — no battles depicted]

Many neurodivergent children have a very limited range of foods they will eat. This is called food selectivity, and families are often told the child is being stubborn, spoiled, or manipulative.

Food selectivity is almost always sensory. The texture of a food, its smell, its visual appearance, its temperature, or the sound it makes when chewed can be genuinely intolerable to a nervous system that processes sensation intensely.

This means that pressure, forcing, or hiding foods rarely works and often makes the range narrower over time by adding anxiety to the mealtime experience.

What does help: keep the mealtime environment calm and predictable. Always include one safe food on the plate. Introduce new foods through exposure without pressure — the food is on the plate but your child is not required to eat it. Over many exposures, anxiety reduces.

Track what your child eats in the daily check-in. If the range is very narrow or nutrition is a concern, a referral to an occupational therapist or paediatric dietitian can help.

[End card: 'Take the battle out of one meal this week. One safe food. No pressure.']

Social Skills

C-20 — Peer play by age — what's developmentally expected

Code: C-20 | **Domain:** Social Skills | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~65 sec

[Show children at different ages playing — accurately staged]

Before worrying about your child's social development, it helps to know what is actually expected at their age.

Under two: solitary play is normal. Children play alongside others but not with them. This is appropriate.

Two to three: parallel play — playing next to another child with similar toys, but not sharing or coordinating. Still normal.

Three to five: associative play begins — some sharing, some interaction, but not yet organised joint play. Children dip in and out of each other's games.

Five and above: cooperative play — shared goals, rules, role negotiation. This is when social differences often become more visible for neurodivergent children, because the peer group is now doing something much more complex.

Neurodivergent children often develop social play on a delayed timeline. Parallel play at age six is not a failure — it is a starting point to build from.

[End card: 'Identify where your child is on this ladder — that is your starting point.']

Social Skills

C-21 — Explaining your child's needs to their school

Code: C-21 | **Domain:** Social Skills | **Audience:** Caregivers | **Age:** All ages | **Duration:** ~70 sec

[Caregiver sitting confidently — friendly, empowered tone]

Advocating for your child at school is one of the most important — and often most exhausting — parts of parenting a neurodivergent child in Kenya.

Before a school meeting: write down three things. One — what your child finds hard at school and why, in plain language. Two — what helps at home that could work at school. Three — one specific thing you are asking the school to do differently.

In the meeting: lead with your child's strengths first. Teachers are more receptive when they understand what your child is capable of, not just where they struggle.

If the school says 'your child just needs more discipline' — stay calm. Say: 'I understand that. I want to share what we have learned about why this happens and what actually helps. Can I show you?'

Bring a copy of your child's weekly summary from the app. It gives you objective data to refer to. You are not just a worried parent — you have a record.

[End card: 'Before your next school conversation — write down your three points.']

Caregiver Hub

C-22 — Progressive muscle relaxation — a guided session

Code: C-22 | **Domain:** Caregiver Hub | **Audience:** Caregivers | **Age:** Adult | **Duration:** ~75 sec

[Soft, calm visual — no child visible. This is explicitly for the caregiver]

This one is for you. Not your child — you.

Sit somewhere you will not be interrupted for two minutes. Feet on the floor.

Hands in your lap.

Take one slow breath in through your nose — [pause 3 sec] — and out through your mouth. [pause 3 sec]

Tighten your hands. Make fists. Hold — two, three — and release. Feel the difference.

Shrug your shoulders up to your ears. Hold — two, three — and release. Let them drop completely.

Gently clench your jaw. Hold — two, three — release. Let your tongue fall away from the roof of your mouth.

One more breath in — [pause 3 sec] — and slowly, fully out. [pause 4 sec]

That is it. Three muscle groups. Two minutes. Cortisol reduced.

Do this before a difficult moment if you can. Or after one, if that is all there was time for.

You are doing enough.

[End card: 'Save this video. Use it when you need it.']

Caregiver Hub

C-23 — Caregiver burnout — recognising it and asking for help

Code: C-23 | **Domain:** Caregiver Hub | **Audience:** Caregivers | **Age:** Adult | **Duration:** ~70 sec

[Honest, direct, warm — no minimising]

Caregiver burnout is real. It is not weakness, laziness, or a lack of love.

It is what happens when a person gives more than they have to give, for too long, without adequate support.

Signs: you feel nothing when you expected to feel something. You are short-tempered in ways that are not like you. You have stopped enjoying things that used to matter.

You are running on routine because feeling is too expensive.

If any of that sounds familiar — your caregiver check-in score in this app matters.

Low scores over multiple days are not just data — they are a signal to take seriously.

Asking for help is not failure. Telling a family member you need two hours.

Telling your doctor you are struggling. Reaching out to a counsellor.

Joining a parent support group — some are listed in the app's directory.

You cannot sustain the care your child needs if you are running on empty.

You matter too. Not instead of your child — alongside them.

[End card: 'If you are struggling — tell someone today.']

Caregiver Hub

C-24 — Building your multidisciplinary team in Kenya

Code: C-24 | **Domain:** Caregiver Hub | **Audience:** Caregivers | **Age:** Adult | **Duration:** ~70 sec

[Show app screen — specialist directory]

Your child's needs are multidimensional. A great paediatrician alone is not enough.

A comprehensive team typically includes: a developmental paediatrician or psychiatrist for diagnosis and medical oversight, an occupational therapist for sensory and motor needs, a speech and language therapist for communication, and a psychologist or counsellor for the family system.

In Kenya, access is real challenge. Not everyone is in Nairobi. Not every county has a paediatric OT. The app's specialist directory lists providers by county, including those offering telehealth consultations.

What to do with limited access: prioritise. Start with whatever specialist can do the most for your child right now — often the OT, because sensory and motor needs touch everything else. Use this app's weekly summary at every appointment so that whoever you see has the full picture.

Document everything. A record of your child's history that travels with you removes the burden of starting over every time.

[End card: 'Check the specialist directory for providers near you.']

Part 4 — Children's Animated Series

Four series, each with a distinct age band, central character, visual style, and therapeutic focus. All episodes are 3–5 minutes. Each ends with an interactive moment the child can do with their caregiver. All episodes should be produced in English and Kiswahili, with Kenyan settings, names, food, clothing, and landscapes.

Animation style guidance: 2D character animation, warm palette, simple backgrounds. Avoid generic 'Western' aesthetics — characters should have Kenyan names, Kenyan homes, Kenyan school uniforms. Jibu (Series A mascot) should be a unique creature with no Western cartoon precedent — consider drawing from Kenyan folklore and nature.

Series A — My Feelings Are Big

Age band: 4–7 years · 6 episodes · 3–5 minutes each

Focus	Emotional regulation, naming feelings, calming strategies
Characters	Amara (5-year-old girl) and Jibu (her small creature companion who lives in her backpack)
Visual style	Warm, expressive, lots of body and face close-ups. Jibu appears to help Amara name and navigate feelings.
End ritual	Each episode ends with Amara showing a feelings chart and asking: 'How does YOUR body feel right now?'

Episode A01 — The Big Feeling

Therapeutic theme: Anger — naming it, feeling it, calming it

[Open: Amara at school. A classmate takes her crayon without asking.]

Amara's face changes. Her fists tighten.

Jibu pops out of her backpack: 'Amara — your face looks like a storm cloud.'

'What is happening in your body?'

Amara: 'My tummy is tight. My hands want to grab.'

Jibu: 'That feeling has a name. It is called ANGRY.'

'It is okay to feel it. It means something important happened to you.'

Jibu holds up three fingers: 'Let's do three breaths together.'

'Breathe in like you're smelling chapati... and out like you're blowing it cool.'

[Animation: Amara's red body softens slightly with each breath]

The tight feeling gets smaller — not gone, but smaller.

Jibu: 'Now — what words can we use?'

Amara walks to the classmate: 'That was my crayon. I was using it. Please give it back.'

[End card: Amara's feeling-face chart appears. 'How does YOUR body feel right now?']

Episode A02 — Too Much Too Fast

Therapeutic theme: Overwhelm — understanding the flooding feeling

[Open: Amara at a birthday party — music loud, lots of people, flashing lights]

[Animation: sound waves shown as large colourful shapes pressing towards Amara]

Amara covers her ears. Her colours turn into swirling reds and oranges.

Jibu appears near her ear: 'Your body is getting very full, Amara.'

'Sometimes everything comes in too fast and the cup overflows.'

[Animation: a simple cup filling — then overflowing]

Amara: 'I don't like it here. There is too much.'

Jibu: 'That is your body telling the truth. Let's find a quieter spot.'

Amara finds a corner. She sits. Jibu sits with her.

'Going to the quiet spot is not giving up. It is being kind to your body.'

[Animation: the colours calm gradually — red fades to soft yellow]

After some time, Amara is ready to go back — a little, on her own terms.

[End card: 'When is YOUR body's cup getting too full? Point to your feeling.']

Episode A03 — Sad Is Okay

Therapeutic theme: Sadness — validating it, not fixing it

[Open: Amara's dog is sick. She is very quiet.]

Jibu: 'You are very quiet today, Amara.'

Amara: 'My heart feels heavy.'

Jibu: 'That is sadness. Sadness is not wrong. It means you love something.'

'You do not have to make it go away.'

Amara: 'But I don't like it.'

Jibu: 'I know. Let it be there. We will sit with it.'

[Animation: Amara and Jibu sit quietly together. Gentle music. No rushing.]

Mama comes and sits beside Amara. No words. Just a hand on her back.

[Animation: Amara's colours slowly shift from heavy grey-blue to soft blue]

Jibu: 'Sadness gets smaller when someone sits with you.'

'You don't have to be happy. You just have to know you are not alone.'

[End card: 'It is okay to feel sad. Who sits with you when you feel sad?']

Episode A04 — Scared in My Belly

Therapeutic theme: Fear and anxiety — where it lives in the body

[Open: Amara about to go to a new school for the first time]

Amara's tummy makes a swirling motion in the animation.

Jibu: 'What is that?'

Amara: 'My tummy is doing something strange. Like butterflies.'

Jibu: 'That is called scared. Or sometimes worried. It lives in the tummy.'

'It is your body saying — I do not know what is coming.'

Amara: 'Will the scared go away?'

Jibu: 'Sometimes. And sometimes it gets smaller when we walk into the thing anyway.'

[Animation: Amara stands at the school gate. Takes a breath. Steps in.]

Jibu walks beside her.

End of day — Amara comes out smiling. The tummy butterflies are quieter.

Jibu: 'You were scared AND you did it. Both things can be true.'

[End card: 'Where in YOUR body does scared live? Touch that spot right now.']

Episode A05 — Happy Looks Different

Therapeutic theme: Joy — that it looks different for different people

[Open: children at play — some laughing loudly, some quiet but smiling]

Amara is spinning in a corner by herself — clearly happy, but alone.

A friend approaches: 'Why are you spinning? That is not playing.'

Amara stops, uncertain.

Jibu: 'Amara — how did spinning feel?'

Amara: 'It felt like sparkles. It feels like happy.'

Jibu: 'Then it IS playing. Your happy might look different. That is allowed.'

[Animation: different children shown feeling joyful in very different ways — one jumps, one is very still and focused, one spins, one hums softly]

Narrator (Jibu): 'Happy is not one shape. Your happy is allowed to be yours.'

[End card: 'What does happy feel like in YOUR body? Can you show me?']

Episode A06 — When I Lose My Calm

Therapeutic theme: Meltdowns — what happens and that it's okay to come back

[Open: Amara has a big overwhelming moment — too much at once]

[Animation: colours explode — fast, bright, chaotic — no words for a moment]

[Gradually, slowly, the colours slow. Amara is on the floor, exhausted.]

Jibu arrives quietly. Sits nearby. Does not speak immediately.

After a moment: 'The storm passed, Amara.'

Amara: 'I couldn't stop it.'

Jibu: 'Sometimes the storm is too big. That is not your fault.'

'You are not bad because you lost your calm.'

Mama comes. Sits next to Amara. Holds her without talking.

[Animation: warmth slowly returns to the colour palette]

Jibu: 'After the storm — you come back. You always come back.'

'And when you do — the people who love you are still here.'

[End card: 'After a hard moment — what helps you come back? Hug? Quiet? Show me.']

Series B — Friends at School

Age band: 5–8 years · 6 episodes · 3–5 minutes each

Focus	Social skills — joining play, friendships, personal space, reading social cues
Characters	Baraka (7-year-old boy who loves trains and finds noisy classrooms hard), his classmates, and his
Visual style	Naturalistic Kenyan school setting. Social situations shown realistically — including misunderstandings
End ritual	Each episode ends with a 'what would YOU do?' moment — two choices shown on screen for the

Episode B01 — How to Join the Game

Therapeutic theme: *Joining group play — the four steps*

[Open: Break time. Baraka watches children play catch from a distance.]

Narrator: 'Baraka loves trains. He knows every train in Kenya.'

'But joining the game? That part is hard.'

[Animation shows Baraka's thinking — a mix of wanting to join and not knowing how]

Narrator: 'Step one — watch first. Learn what they are playing.'

[Baraka watches. Animation labels: 'Catch. Taking turns. Three players.']

Narrator: 'Step two — move a little closer. Not too close. One arm's length away.'

[Baraka steps forward. Not crowding. Just nearby.]

Narrator: 'Step three — wait for a pause. Not when someone is about to throw.'

[Baraka waits. A natural gap appears in the game.]

Narrator: 'Step four — two words. Can I play?'

Baraka: 'Can I play?'

Children: 'Yes! You can be on this side.'

[Baraka smiles — small but real]

Narrator: 'Sometimes they say yes. Sometimes they say not right now.'

'Both happen. And Baraka is still okay either way.'

[End card: 'What would YOU do? Wait and watch — or ask right away?']

Episode B02 — My Space, Your Space

Therapeutic theme: *Personal space — understanding it and respecting it*

[Open: Baraka stands very close to a classmate who looks uncomfortable]

Narrator: 'Baraka likes to be close when he is talking. It feels normal to him.'

'But his classmate Zawadi looks different.'

[Animation: Zawadi shifts away. Baraka moves closer. Zawadi shifts again.]

Ms. Wanjiru comes over. She draws an invisible circle around Zawadi.

Ms. Wanjiru: 'Everyone has a bubble of space around them. About one arm's length.'

'When we go inside someone's bubble without asking — it can feel uncomfortable.'

[Animation: 'bubbles' appear around all children in the class]

Baraka tries again — this time staying at arm's length.

Zawadi smiles this time. Stays still.

Narrator: 'Personal space is different for everyone. Some bubbles are bigger.'

'When you are not sure — leave more space, not less.'

[End card: 'Show me how big YOUR bubble is with your arms right now.']

Episode B03 — When They Don't Want to Play

Therapeutic theme: Rejection — handling it without shutting down

[Open: Baraka approaches two classmates who are playing a game he likes]

Baraka: 'Can I play?'

Classmate: 'Not right now. We only have two players for this game.'

[Animation: Baraka's body slumps. Colours go grey.]

Narrator: 'That felt bad. It was supposed to feel bad — being left out is hard.'

Baraka walks to a bench and sits down.

Jibu [appearing as a thought in his mind]: 'What can we do with this feeling?'

Baraka thinks: 'I could watch. Or find someone else. Or play alone for a bit.'

[Animation: three doors appear — each with an option]

Baraka chooses 'play alone for a bit.' He gets out his small train model.

Another classmate sees the train: 'Whoa — is that the SGR engine?'

Baraka lights up: 'Yes. Want to see how it works?'

[A new connection begins — one Baraka did not expect]

Narrator: 'Being told no is hard. But it is not the end of the story.'

[End card: 'When someone says not right now — what would YOU do?']

Episode B04 — What Does That Face Mean?

Therapeutic theme: Reading facial expressions — with Kenyan expressions modelled

[Open: Baraka is telling a long story about trains. His friend's face changes.]

Narrator: 'Baraka is very excited. He has so much to tell.'

[Animation: friend's eyes look to the side, eyebrows lower slightly]

Narrator: 'Did you see that? Baraka did not notice.'

Ms. Wanjiru pauses the class for a 'face game.'

She shows three faces: happy / bored / confused

Children guess each one. [Correct answers animated with checkmarks]

Ms. Wanjiru: 'When someone looks away when you are talking — what might that mean?'

Children: 'They are bored? Or something distracted them?'

Ms. Wanjiru: 'Yes. And if you are not sure — you can ask.'

Baraka tries it: 'Am I talking too much about trains?'

Friend laughs: 'A little. But I like the part about the bridges.'

Narrator: 'You do not have to guess every face perfectly.'

'But when you are not sure — you are always allowed to ask.'

[End card: 'What does THIS face mean?' [Show a gentle curious expression]]

Episode B05 — Sharing Is Hard

Therapeutic theme: Turn-taking and sharing — why it is hard and how to practise

[Open: Art class. One pair of scissors for two children.]

Baraka grabs the scissors. He needs them RIGHT NOW.

Classmate: 'Hey! I was using those!'

[Both hold the scissors. Standoff.]

Ms. Wanjiru: 'Let's think about this. Who had them first?'

Baraka: '...She did.'

Ms. Wanjiru: 'So what is the fair thing?'

Baraka thinks. It is hard. He really wants them now.

Ms. Wanjiru: 'Fair does not always mean comfortable. It means everyone gets a turn.'

[Animation: a simple timer appears — two minutes each]

Baraka waits. The two minutes feel very long.

[Animation: clock face shown honestly — time passing slowly]

Then it is his turn. He cuts exactly what he needed.

Narrator: 'Waiting for your turn is hard. Almost always. That is not wrong.'

'But it is something we can practise — and it gets a little easier each time.'

[End card: 'What helps YOU wait for your turn? Tell me or show me.']

Episode B06 — A Real Apology

Therapeutic theme: Repairing — how to give a meaningful apology

[Open: Baraka, frustrated, says something unkind to a classmate]

[Classmate's face falls. She walks away.]

Narrator: 'Baraka said something he did not mean — or maybe he meant it in the moment.'

'Either way, it hurt someone.'

Baraka knows something does not feel right.

Jibu [appearing as a thought]: 'What happened?'

Baraka: 'I said something mean. She looked sad.'

Jibu: 'Do you want to fix it?'

Baraka: 'How?'

Jibu: 'A real apology has three parts. What you did. That you are sorry it hurt.'

'And what you will try to do differently.'

[Animation: three speech bubbles appear with each part]

Baraka approaches the classmate:

'I said that thing that was mean. I am sorry it hurt you. I will try to stop before I speak next time.'

[Classmate considers. Then nods.]

Narrator: 'A real apology does not always fix everything. But it is always worth giving.'

[End card: 'What are the three parts of a real apology? Can you remember them?']

Series C — My Body Knows

Age band: 3–6 years · 5 episodes · 3–5 minutes each

Focus	Sensory awareness — learning to notice and communicate body signals
Characters	Tatu (4-year-old girl), her body shown in animation as a simple glowing figure, Mama
Visual style	Very simple, minimal dialogue, strong visual metaphors. Body states shown as colour changes ins
End ritual	Each episode ends with: 'What is YOUR body telling you right now?' Caregiver prompt: 'Ask your c

Episode C01 — Too Loud, Too Much

Therapeutic theme: Auditory overload — recognising it, naming it, finding calm

[Open: Tatu at a noisy birthday party]

[Animation: sound waves shown as large shapes pressing in from all sides]

Tatu presses her ears with her hands.

Narrator (soft, slow): 'Tatu's ears feel very full.'

'The music is big. The voices are big. Everything is BIG.'

[Animation: Tatu's body outline fills with orange-red at the ears and head]

Narrator: 'Her body is saying: this is too much for me right now.'

Mama sees Tatu. She does not make Tatu go back in.

They find a quiet corner outside.

[Animation: the colours soften slowly. Orange becomes yellow. Yellow becomes light blue.]

Narrator: 'The quiet corner is not giving up. It is being kind to your body.'

[Tatu breathes. Colours calm further. She looks at Mama.]

[Later, she goes back inside — on her own terms. For a short time.]

Narrator: 'Your body talks to you. Listening to it is a superpower.'

[End card: 'What is YOUR body telling you right now? Point to where.']

Episode C02 — My Body Needs to Move

Therapeutic theme: Proprioceptive and vestibular need — why some bodies need more movement

[Open: Tatu is sitting at the table, but her body keeps moving]

[She wiggles, gets up, comes back, wiggles more]

Narrator: 'Tatu's body is asking for something.'

[Animation: Tatu's body outline shows a buzzing, active energy — small sparks]

Narrator: 'Some bodies need more movement to feel settled.'

'It is not naughty. The body is asking — please move me.'

Mama says: 'Tatu — let's do five big jumps before we sit down.'

[They jump together. Five big jumps. Animation shows sparks calming]

Tatu comes back to the table. Sits. The buzzing is quieter.

Narrator: 'Moving first helps some bodies settle.'

'Your body knows what it needs. It just cannot use words yet.'

[End card: 'Does YOUR body need to move right now? Let's find out together.']

Episode C03 — This Feels Scratchy

Therapeutic theme: Tactile sensitivity — clothing, textures, and the body's response

[Open: Tatu and Mama getting dressed. Tatu pulls off a shirt immediately.]

Mama: 'What is wrong?'

Tatu: 'It hurts. It feels scratchy.'

[Animation: the shirt fabric shown magnified — rough fibres touching Tatu's skin]

[Tatu's body outline goes red at the shoulders and arms]

Narrator: 'For Tatu, that fabric feels very rough. Like sandpaper.'

'Other people might not notice. But Tatu's skin is very sensitive.'

'This is real. It is not pretending.'

[Mama looks for a softer option — a cotton shirt with no tags]

[Tatu puts it on. The red in her body outline fades.]

Narrator: 'Bodies are different. Some skin needs softer things.'

'When your child says something hurts — believe the body.'

[End card: 'Does anything on YOUR body feel scratchy or uncomfortable right now?']

Episode C04 — Hungry or Something Else?

Therapeutic theme: Interoception — noticing internal body signals like hunger and thirst

[Open: Tatu is grumpy and Mama cannot figure out why]

Mama: 'What is wrong, Tatu?'

Tatu: 'I do not know.'

[Animation: Tatu's body has a question mark inside it]

Narrator: 'Sometimes the body sends signals — but we have not learned to read them yet.'

Mama: 'Let's check. Is your tummy empty?'

[Tatu pauses. Puts her hand on her tummy. Thinks.]

Tatu: '...Yes.'

[Animation: a small bowl in the tummy area — empty, then filling]

They eat a snack together. Tatu's body colours shift from grey-yellow to warm orange.

Tatu: 'I feel better.'

Narrator: 'Hunger was the signal. Food was the answer.'

'Learning to read your body is a skill. We practise it every day.'

[End card: Caregiver prompt: 'Let's check YOUR body signals together. Hungry? Thirsty? Tired?']

Episode C05 — My Safe Place

Therapeutic theme: Regulation — having a go-to calming space and what to do there

[Open: Tatu's cosy corner — a mat, soft cushion, one familiar toy]

Narrator: 'This is Tatu's safe place.'

'It is not a punishment place. It is a calm-down place.'

[Show Tatu going there after a hard moment — walking in, not being sent]

Narrator: 'Tatu has learned: when my body feels too full, I can go to my safe place.'

[Animation: Tatu's swirling colours calm as she settles on the cushion]

Tatu squeezes a small weighted toy. Breathes.

Jibu [as a thought]: 'What do you need right now?'

Tatu thinks: 'Quiet. And my favourite blanket.'

[She pulls the blanket over her lap. The colours calm fully.]

Narrator: 'You are always allowed to go to your safe place.'

'It is one of the kindest things you can do for your body.'

[End card: 'Where is YOUR safe place? Let's find or make one together.']

Series D — My Day, My Way

Age band: 4–8 years · 5 episodes · 3–5 minutes each

Focus	Routines, predictability, transitions, and building independence
Characters	Kofi (6-year-old boy), his family, his visual schedule on the wall
Visual style	Practical and warm. The visual schedule is a character in its own right — shown prominently in ea
End ritual	Each episode ends with Kofi checking off his schedule. 'What is on YOUR schedule today?'

Episode D01 — Morning with Kofi

Therapeutic theme: Morning routines — how a schedule makes mornings calmer

[Open: Kofi's visual schedule on the wall — 5 pictures in order]

Narrator: 'Kofi's morning has a special order.'

'When things happen in order, Kofi's brain feels calm.'

[Wake up → schedule check → breakfast → dress → school bag → leave]

[Each step shown with a satisfying tick sound as Kofi completes it]

Narrator: 'Kofi knows what is coming. His brain does not have to wonder.'

[One morning: Mama changes the order unexpectedly — breakfast is not ready]

[Kofi's colours shift. His body stiffens.]

Narrator: 'Kofi's brain expected: breakfast first. Then dressing.'

'When things change, it feels like a bump in the road.'

Mama shows him the schedule. She moves the picture cards: dressing moves before breakfast.

'New plan today, Kofi. See? Dress first. Then we eat.'

[Kofi looks at the updated schedule. His colours settle.]

Narrator: 'A picture of the new plan helps Kofi's brain understand.'

'Then the bump gets smaller.'

[End card: 'What is on YOUR morning schedule? Let's make a list together.']

Episode D02 — First/Then

Therapeutic theme: The first/then system — making demands predictable

[Open: Kofi does not want to tidy his toys]

Mama holds up a simple card: two boxes. A drawing of toys. An arrow. A drawing of football.

Mama: 'First tidy. Then football.'

[Kofi looks at the card. He understands what is coming.]

Narrator: 'The first/then card shows Kofi: there is something good on the other side.'

[Kofi tidies — not happily, but steadily]

[The toys are away. Mama ticks the first box on the card.]

Mama: 'First is done. Now — then.'

[They go outside. Football.]

Narrator: 'First/then works because it makes the deal visible.'

'Kofi does not have to trust words alone. He can see the plan.'

[End card: Mama makes a first/then card with the viewer. 'What is YOUR first/then today?']

Episode D03 — The Warning Bell

Therapeutic theme: Transition warnings — giving the brain time to switch

[Open: Kofi is deeply absorbed in building something — very focused]

[Without warning, Baba says: 'We are going. Now. Get your shoes.']

[Kofi crashes — he is not ready. His building is not finished. His brain is still there.]

Narrator: 'Kofi's brain was somewhere else. It needed time to come back.'

[Next day — Baba tries the warning system]

Baba [5 minutes before]: 'Kofi — five more minutes, then we go.'

[Kofi keeps building — but a small part of his brain has heard]

Baba [2 minutes]: 'Two more minutes.'

[Kofi starts to slow down. He puts the last piece on.]

Baba: 'One minute. Let's get your shoes ready.'

[Kofi goes to his shoes without a meltdown]

Narrator: 'The warning gives the brain time to finish.'

'Five minutes, two minutes, now — that is the system.'

[End card: 'What helps YOUR brain get ready to stop? Tell me.']

Episode D04 — When Plans Change

Therapeutic theme: Unexpected changes — coping strategies

[Open: Kofi has been looking forward to visiting his grandmother all week]

[The morning of the visit — it rains hard. The road is flooded. They cannot go.]

[Kofi's face — devastation. This is a real loss, shown honestly.]

Narrator: 'Plans change sometimes. Even important ones. Even ones we were very excited about.'

[Kofi cries. Mama sits with him. Does not rush the feeling.]

Mama: 'I know. It is really disappointing. It is okay to be upset.'

[After the cry — Mama brings out the schedule]

'The visit is moved to next Saturday. See — here it is.'

[The visit card is moved on the schedule to next week]

Narrator: 'Seeing the new plan helps. But first, the feeling needed space.'

Kofi: 'Will it really be next Saturday?'

Mama: 'Yes. I will put it right here so we can both see it.'

[Kofi touches the card. Still sad. But a little more settled.]

Narrator: 'When plans change — feel it first. Then see the new plan.'

[End card: 'What do YOU do when something disappointing happens?']

Episode D05 — I Did It Myself

Therapeutic theme: Independence — the pride of doing something alone

[Open: Kofi's schedule has a new picture — putting on his shoes independently]

Narrator: 'Kofi has a new goal on his schedule.'

[Kofi looks at the picture of shoes. He looks at his shoes. He tries.]

[His fingers struggle with the straps. He tries again. And again.]

[Mama is nearby but does not help unless he asks]

[The strap clicks. Both shoes are on.]

[Kofi looks up — his face says everything]

Kofi: 'Mama. I did it.'

Mama: 'You did. All by yourself.'

[Animation: Kofi's body colours glow warm gold for a moment]

Narrator: 'Learning new things takes time. And some attempts.'

'But the moment of doing it yourself — that belongs to you.'

[Kofi goes to his schedule and ticks the shoes picture. With great ceremony.]

[End card: 'What is something YOU can do all by yourself? Tell me. I want to know.']

Part 5 — Clinical Report Template

5.1 Weekly Automated Summary — Sample Layout

The following layout is generated automatically every Sunday and exported as a formatted PDF the caregiver can share via WhatsApp or email. All fields are populated from the daily check-in. The summary is designed to be read in under two minutes by a clinician.

NEURODIVERGENT CARE APP — WEEKLY SUMMARY	
Child name	[As registered in app]
Week ending	[Date]
Prepared by	[Caregiver name]
Shared with	[Clinician / MDT]
SLEEP QUALITY (7 days)	
Days: easy / long / disrupted	e.g. 2 / 3 / 2
Notable pattern	Auto-generated plain-English note
Flag	Amber if 3+ disrupted nights
SENSORY DIFFICULTY (7 days)	
Days: none / mild / significant / severe	e.g. 1 / 2 / 3 / 1
Notable pattern	Auto-generated plain-English note
Flag	Red if 2+ severe days
MOOD ON WAKING (7-day average)	e.g. 2.8 / 5
EATING PATTERN	e.g. Selective 4 days, refused 1 day
CAREGIVER COPING (7-day average)	e.g. 2.1 / 5 [Flag if below 2.5]
CAREGIVER FREE TEXT (all 7 entries)	
Monday	e.g. Hard morning — sensory meltdown before school
Tuesday	e.g. Better evening. He used the calming corner himself.
Wednesday – Sunday	[Additional entries listed chronologically]
AUTOMATED RED FLAG SUMMARY	

	e.g. Two severe sensory days recorded. Caregiver coping score below threshold for 4 days
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5.2 Multidisciplinary Team Routing Guide

The weekly summary can be routed to different members of the child's care team. Use the following guide to determine which sections are most relevant for each specialist:

Specialist	Priority fields in summary	Red flag triggers
Occupational therapist	Sensory difficulty frequency, free text (sensory), child	2 or more sensory days, refusal 3+ days
Developmental paediatrician / psychiatrist	5-day summary — all fields. Mood trend, caregiver coping	Caregiver coping below 2.5 for 4+ days; mood b
Speech & language therapist	Caregiver free text (communication entries), child need	Communication-related entries flagged by careg
Neurologist	Sleep quality trend, severe sensory frequency, any care	Severe or disrupted sleep days, caregiver note o
Psychologist / counsellor	Caregiver coping trend, caregiver free text, child mood	Caregiver coping below 2 for any 3 days in a we
School / class teacher	Weekly mood summary, sensory difficulty frequency, 3 or	Significant sensory days that may affect scho

This document is a living work plan. Scripts should be reviewed with a speech and language therapist and a clinical psychologist or psychiatrist before production to ensure therapeutic accuracy. Kiswahili adaptations should be done by a native speaker with child development knowledge — direct translation is not recommended.

For questions about clinical content, contact Dr. Kendi. For production questions, refer to the separate production brief. Version 1.0 — 2025.